

Patient ID: Harris

Period: May 1 to June 1, 2026

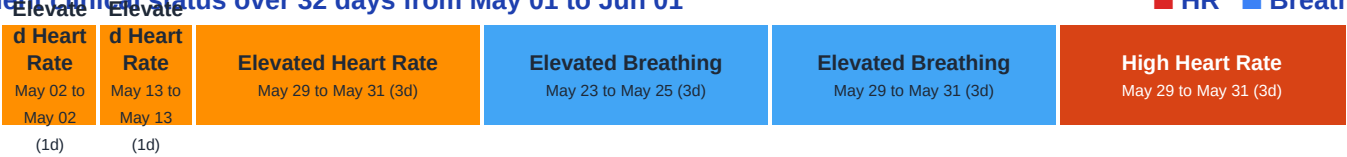
Report Date: June 2, 2026

Coverage: 742/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing



Last 24 Hours

Vital		Avg		Min		Max			
Heart Rate		88 bpm		48 bpm		115 bpm			
Breathing		19 brpm		9 brpm		47 brpm			
Episodic Events		Total Hours		Highest Burden		Episodes/day		Coverage	
24		35h		Elevated Breathing		<1		100%	

Major Findings: Sustained elevated breathing (avg 25, peak 51)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 29	4 AM to 6 AM	7h	3	High Heart Rate	104	84	115	Check temp, hydration, pain; review for arrhythmia or infection
May 02	1 AM to 2 AM	4h	2	Elevated Heart Rate	97	70	108	Check temp, hydration, pain; review for arrhythmia or infection
May 13	3 AM to 4 AM	2h	2	Elevated Heart Rate	96	59	105	Check temp, hydration, pain; review for arrhythmia or infection
May 23	2 AM to 3 AM	9h	7	Elevated Breathing	25	15	47	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 29	3 AM to 4 AM	6h	5	Elevated Heart Rate	97	59	108	Check temp, hydration, pain; review for arrhythmia or infection
May 29	4 AM to 6 AM	5h	3	Elevated Breathing	25	14	47	Check SpO2 and lung sounds; assess for fluid overload or early infection

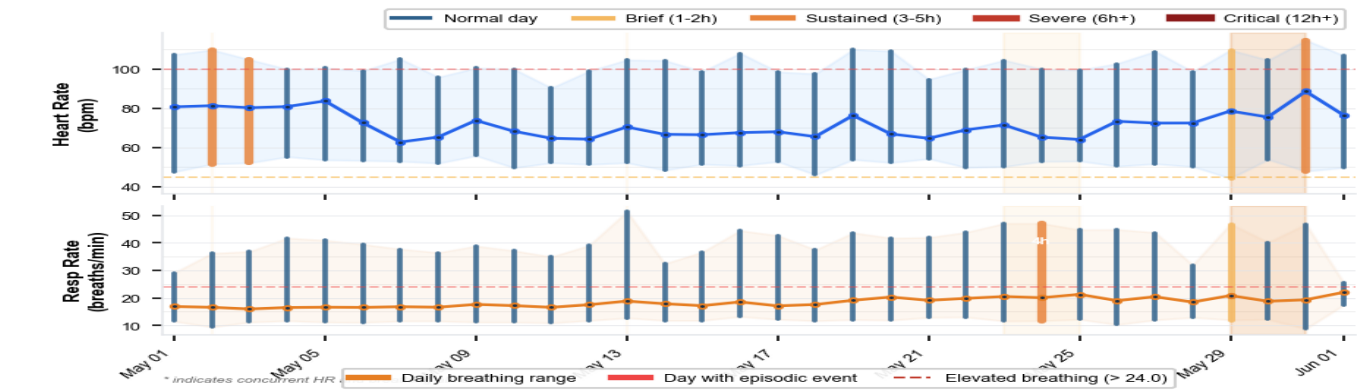
The P5 to P95 heart rate spread was 33 bpm (59 to 92), indicating significant cardiac variability. Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 47 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 104 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 97 bpm, peak 108 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Overall trajectory shows 6 unstable phases with 33 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

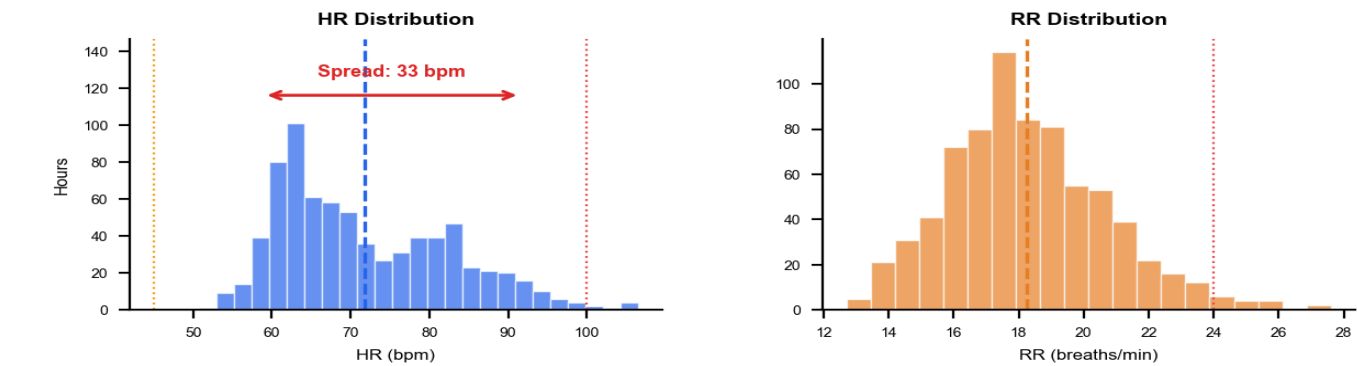
Clinical Pattern Observations:

- 1. High variability: HR spread 33 bpm (58 to 91).
- 2. Clustered pattern: Episodic events on 5 of 32 days (15%).
- 3. Nocturnal pattern: 5 of 6 eligible episodes during evening or night hours.

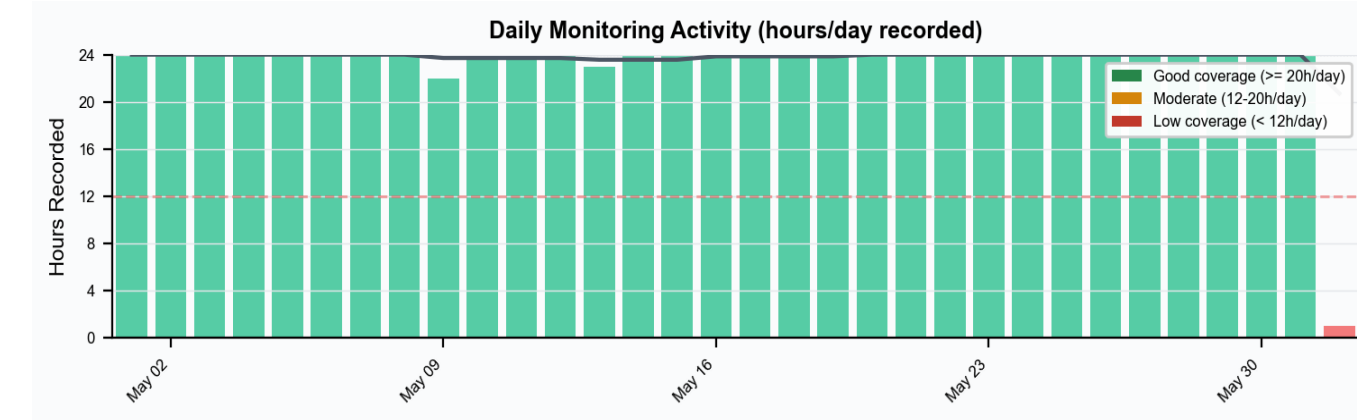


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 bpm
- High Breathing > 30 bpm
- Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.